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Name: ضحي احمد محمد اسماعيل National ID: 27811150106044	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Plus Goserline
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 بطاقة تحقيق الشخصية

ضيفي
 احمد محمد السماعيل
 ١٧ ش الجدي حلقى حوران
 المعصرة - القاهرة

٢٧٨١١١٥٠١٠٦٠٤٤
 ٢٧٨١١١٥٠١٠٦٠٤٤
 I08648077

٢٠٢١/٠٣ ٢٧٨١١٥٠١٠٦٠٤٤
 حاصلة على بكالوريوس خدمة اجتماعية
 أنش
 خالد محمد شاكر زكي الشانور
 البجاية سارية حتى ٢٠٢١/٠٣/١٤

مصرية
 مصرية

٢٠٢١/٠٣



Name: ضحي احمد محمد اسماعيل		
MR. : 3366		
Detailed Medical Report		
		السيد
تحية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was evaluated for severe back pain and a request for a bone scan while on ARIMIDEX. A bone scan performed on 25 March demonstrated tracer uptake at L5/Sacrum and the right sacroiliac articulation, prompting recommendation for MRI; the patient was continued on ARIMIDEX and calcium supplementation, with a plan for ZOLADEX 10.8 SC on 22 April and full laboratory workup. The treatment plan was updated to include ARIMIDEX plus supplementation, with ZOLADEX scheduled for a three-month interval on 15 July, and the pharmacy was instructed to prepare the medication for the next visit on 23 August. The current status is that the patient remains on ARIMIDEX with planned ZOLADEX administration.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	الطبيب المعالج : نهى سعيد علي محمد	
يعتمد ,		

MDT Form

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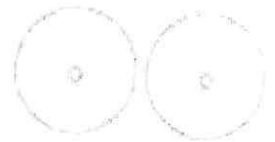
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Breast Imaging:

Clinical data

Mets Workup:

CAP CT: no mets-axillary LN

DX

ES

PR

Iter 2 Neu

SISH

Ki67

IDC

8/8

7/8

0

None

Y

3

N

3

M

0

Sonamammography

PATIENT HISTORY

locally advanced RL breast cancer

received EC x3 + taxotere x2 only then lost FU last on 2/5/2023

new CAP CT: stationary, no mets

MDT OUTCOME & PLAN

pt. lost FU for 7 months (refused surgery), now showing mild disease progression.

After extensive pt. counselling pt. agreed to Rt. MRM

MEDICAL ONCOLOGY

SURGERY

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

Patient Name : **مريم احمد محمد علي**
Patient No : **27811133100044**
Ref No : **640/24**
Sex : **Female**
Age : **45 Y**
Referring Doc :

Request No : **13007672**
Request Date : **17/02/2024**
Verification Date : **04/03/2024**
Financial Category : **شخص - فئة الاولى**
Department : **Out Patient**
Registered by : **Ali mostafa**

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination

- Procedure: Right MRM measured 22x18x9cm with covering skin ellipse measured
- Axis 1cm
- Tumor extent: A greyish white firm tumor bed side.
- Size: 7x5x4 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins: 1.5 cm away from deep margin.
- Lymph nodes number and size: 12 L.Ns up to 1.7x1.3cm.

Microscopic Examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II.
- Maximal tumor diameter 7cm
- Actual tumor cellularity: 50%, tumor necrosis index: 50%
- Ductal carcinoma in situ: No
- Skin involvement: Free
- Lymph node involvement: Metastatic deposits 6/12
- Resection Margins: Free
- Histologic response to therapy: Pathological partial response
- Microcalcifications: No

Diagnosis:

Right breast cancer post NACT, MRM specimen:
Invasive duct carcinoma, grade II.

Verified by:

Dr Mohamed Torky

Printing Date : 06/03/2024 10:43:12

Page Tot 2

Printing By : Ali mostafa

CAP

مركز الاحترافي للصحة النسائية - مختبر أمراض النساء - القاهرة
Boulevard - First Floor-Building No. 937, Nile Corridor - Mid El Ghardiga - 11110, Egypt

Referring Physician : د. محمد عبد الحليم
Patient No. : 27811150405244
Path No. : 646/24
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13007672
Request Date : 17/02/2024
Verification Date : 04/03/2024
Financial Category : فئة المرضى - Out Patient
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

- Maximal tumor diameter: 7cm
- Actual tumor cellularity: 50%, tumor necrosis index: 50%
- Resection Margins: Free
- Skin: Free
- Lymph nodes: Metastatic deposits 6/12
- Histologic response to therapy: Pathological partial response
- Stage: ypT3, ypN2
- RCB: 4.472, RCB class III

Signature
Dr. Rana Fathy
Dr. Muhammed Torkey

Verified by
Dr. Mohamed Torkey

Printing Date : 06/03/2024 16:43:12

Page 2 of 2

Printing By : ali mostafa

Patient Name	: الأستاذ / د. محمد أحمد محمد	Request No.	: 13000993
Patient No.	: 27811150106044	Request Date	: 23/11/2022
Path No.	: 3530/22	Verification Date	: 05/12/2022
Sex	: Female	Financial Category	: برنامج التأمين - فئة الأولى
Age	: 44 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA
Right breast mass

NATURE OF SPECIMEN
Tru cut biopsy, collected at 23/11/2022 , 1:19 pm

GROSS EXAMINATION
Multiple grayish white tissue cores up to 1.5 cm , totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

MICROSCOPIC EXAMINATION
* Histological type: Invasive lobular carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* LCIS: Focal
* Angiolinvasion: Couldn't be detected in the received biopsy.
* Perineural invasion: No.
IPT:
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 50% of tumor cells (Allred score 7/8)
Her-2: No membrane reaction in the tumor cells (score 0).
Ki 67: 12 % nuclear reaction in tumor cells

DIAGNOSIS
Right breast mass , Tru cut biopsy.
Invasive lobular carcinoma, Grade II with focal LCIS.
IHC

Verified by :
Dr. Marwa mahmoud

Printing Date : 14/12/2022 09:48:56

Page 1 of 2

Printed By : marwa mohamed youssef

Patient Name الأستاذة / د. منى أحمد محمد
Patient No. 27811150106044
Path No. 3530/22
Sex Female
Age 44 Y
Referring Doc.

Request No. 13000993
Request Date 23/11/2022
Verification Date 05/12/2022
Financial Category برنامج التأمين الصحي
Department Out Patient

Registered by marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

ER: Positive (score 8/8)
PR: Positive (score 7/8)
HER-2: Negative (score 0)
Ki67: Positive in 12% of tumor cells.

Signature
Prof. Dr. Lobna Omar
Dr. Amgen Ahmed

Verified by
Dr. Marwa mahmoud

Printing Date: 14/12/2022 09:49:55

Page 2 of 2

Printed By: marwa mohamed youss

Patient Name: Doha Ahmed Mohamed Ismail	Date : 18/12/2022
Exam : C.T CHEST WITH CONTRAST	Age : 44 Year(s)
Medical ID:	Pat. ID: 25393

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Prominent right axillary lymph nodes are noted the largest it seen measuring about 1.4 x 0.9 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged in size with homogenous low CT texture. Right hepatic lobe segment V focal lesion is seen showing irregular nodular peripheral enhancement which filled in the delayed phase measuring about 3.8 x 2.7 cm. Another smaller enhanced lesion is seen in segment VI measuring about 7 mm in diameter, likely representing hemangioma.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.

Patient Name: Doha Ahmed Mohamed Ismail	Date : 18/12/2022
Exam : C.T CHEST WITH CONTRAST	Age : 44 Year(s)
Medical ID:	Pat. ID: 25393

➤ No abdominal or pelvic collections.

Opinion:

- Known case of breast cancer showing prominent right axillary lymph nodes.
- No pulmonary metastatic deposits.
- Right hepatic lobe focal lesions likely representing hemangioma.

Dr. Ahmed M. Rozeka, MSc

~~Dr. M. Gomaa, M.D.~~

